

SOLACE · USER RESEARCH

Patient & Caregiver Personas

Four composites grounded in psycho-oncology evidence,
representing the full breast cancer care continuum.

Priya, 34

Newly diagnosed

Linda, 52

Active treatment

Margaret, 67

Survivorship

James, 46

Caregiver



Priya

34 · Indian Singaporean

Stage II · HER2+

Pre-menopausal

3 weeks post-diagnosis

NEWLY DIAGNOSED

BACKGROUND

Product manager, married, mother of a 2-year-old. Diagnosed three weeks ago after finding a lump herself. Treatment plan is being finalised; oncologist has raised fertility preservation as an urgent decision that must be made within days.

"I have four days to decide whether to freeze my eggs before chemo starts. I can't think straight. I don't know what question to even ask."

SOLACE FEATURES USED

Appointment Support

Body & Self

Hear From Others

Find Calm

CORE EMOTIONAL NEEDS

Urgency containment

Slow down the panic around a time-pressured decision without dismissing its reality.

Fertility grief support

Space to process what treatment means for her family plans, separate from the medical decision itself.

Appointment preparation

Build specific questions for her oncologist before the fertility preservation window closes.

Peer witness

Hear from young mothers who have been in this exact situation and come through it.

CLINICAL RISK FLAGS

Acute decision-making distress under extreme time pressure

Information overload combined with anticipatory grief

Cultural pressure to be strong for family; actively concealing distress

DESIGN NOTE

Priya's entry into Solace is most likely Appointment Support — the worry-to-question converter addresses her immediate need. Within sessions she will pivot into Body & Self as fertility grief surfaces. She is unlikely to self-identify as needing crisis support, but Find Calm may be needed at night when the decision weight compounds.



Linda

52 · Chinese Singaporean

Stage III · Triple negative

Cycle 4 of 6

Chemo + radiation

ACTIVE TREATMENT

BACKGROUND

Secondary school vice-principal, divorced, two adult children living abroad. Known among colleagues for composure and competence. Has lost her hair; struggles to recognise herself in the mirror. Her children want to visit but she keeps telling them to wait — she does not want them to see her like this.

"I have never in my life not been in control. I don't know who I am right now. I look in the mirror and I don't recognise myself."

SOLACE FEATURES USED

Body & Self

Find Calm

Appointment Support

Hear From Others

CORE EMOTIONAL NEEDS

Identity anchoring

Finding who she is when the roles that always defined her — competent, composed, capable — are stripped away.

Body image support

Hair loss, weight changes, and visible treatment effects during active chemotherapy.

Permission to show distress

Undoing lifelong self-sufficiency as the default coping strategy; learning it is safe to not be okay.

Communication scripts

Language for letting her children in without worrying them, on her terms.

CLINICAL RISK FLAGS

High identity disruption — loss of control is the core psychological wound

Social isolation via self-protective concealment from adult children

Elevated depression risk in triple-negative subtype

DESIGN NOTE

Linda will not present as distressed in clinical settings — her composure is load-bearing. Solace is the first context where she does not have to perform wellness. Body & Self is the natural primary entry point. The design must not be alarming or heavy; she needs to feel she is choosing depth, not being pushed into it.



Margaret

67 · Malay Singaporean

Stage I · ER+

Lumpectomy + radiation

18 months post-treatment

SURVIVORSHIP

BACKGROUND

Retired civil servant, widowed, active in her mosque community. Treatment was caught early; everyone considers her one of the lucky ones. Now on tamoxifen. Has scanxiety before every surveillance visit and has withdrawn from community activities without explaining why.

"Everyone said 'you're done, you're cured.' But I don't feel done. My scan is in six weeks and I haven't slept properly in a month. I feel ungrateful for not being more okay."

SOLACE FEATURES USED

Appointment Support

Find Calm

Hear From Others

Body & Self

CORE EMOTIONAL NEEDS

Survivorship validation

Naming clearly that treatment ending does not mean emotional processing is finished.

Scanxiety management

Tools for the weeks before surveillance appointments, when fear of recurrence is highest.

Body reconnection

Post-lumpectomy changes and tamoxifen side effects on energy, joints, and intimacy.

Peer witness

Others who understand survivor guilt and the specific grief of being told you are lucky.

CLINICAL RISK FLAGS

Survivorship-phase distress invisible to the clinical system post-discharge

Social withdrawal masked by performing recovery for her community

Tamoxifen side effects undisclosed to GP — fatigue, joint pain, libido loss

DESIGN NOTE

Margaret is the persona the oncology system loses sight of entirely. She is discharged, considered cured, and has no clinical touchpoint for emotional support. Her entry into Solace is likely Appointment Support before a surveillance scan, or Hear From Others. The design must not require her to identify as struggling — she needs to be able to start small.



James

46 · Chinese Singaporean

Husband of patient (Linda)

Working full-time

Primary caregiver + sole parent

CAREGIVER

BACKGROUND

Finance manager, married to Linda (Stage III). Running the household alone, managing two adult children's worry from abroad, maintaining his job, and accompanying Linda to treatments. Has told no one at work. His social circle is asking after Linda; no one is asking after him.

"Everyone keeps asking how she is. No one asks how I am. I think they assume I'm not allowed to not be okay. Maybe I assumed the same thing."

SOLACE FEATURES USED

For Caregivers

Find Calm

Hear From Others

Appointment Support

CORE EMOTIONAL NEEDS

Permission to feel

Explicit normalisation of caregiver ambivalence — fear, resentment, grief, and guilt about all of these.

Micro-restoration

60–90 second practices that fit between work calls and hospital visits, not 20-minute meditations.

Communication scripts

Language for the conversation with Linda he cannot start: 'I am scared too.'

Anticipatory grief support

Holding hope and fear about prognosis simultaneously, without a safe outlet.

CLINICAL RISK FLAGS

Caregiver distress invisible — systematically excluded from oncology support

Role strain: sole emotional and domestic anchor with no relief

Anticipatory grief with no naming or processing context

DESIGN NOTE

James will not self-refer to mental health support — he does not think his needs qualify. The For Caregivers feature must be designed so he can enter without having to call himself a caregiver in distress. The word 'caregiver' in the button label is deliberate — it is a functional identifier, not a clinical label. He just needs to recognise himself.

All four personas · Feature usage overview

How each persona engages with Solace's five core features

